

India's Trusted Health Test @Home Service



NABL
ACCREDITED



Booking ID : 16941687185

Sample Collection Date : 17/Feb/2026

Suman Gupta

Female, 61 Yrs

Healthians
Relationship
Status

Preferred
Bronze
Since
2023

A Comprehensive Health Analysis Report

AI Based Personalized Report for You

Flagship Lab in Gurugram

CAP
ACCREDITED
COLLEGE of AMERICAN PATHOLOGISTS
CAP #: 9019582 | AU-ID: 2107746



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

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MACHINE & QC DETAILS

LABORATORY EQUIPMENT DASHBOARD

Suman Gupta

Booking ID : 16941687185



Beckman AU5800

Beckman Coulter : Beckman AU5800

Department - Biochemistry

No. of test(s) performed - 37

Performed Test(s): **RA Test Rheumatoid Arthritis Factor, Quantitative, Liver Function Test, Iron Studies, Kidney Function Test Advance, Lipid Profile**

Description: The Beckman Coulter clinical chemistry analyzer manufactured in USA, is the fastest in the industry, offering a throughput of 2,000 tests per hour using spectrophotometry and 2900 tests per hour with ISE. It utilizes advanced spectrophotometric technology to ensure high accuracy and reliability, delivering over 21 parameters that meet Six Sigma performance standards.

Frequency of Quality Control (QC) : **3 times a day** (Industry best and as per NABL guidelines)



Siemens Atellica IM 1600

Siemens Healthineers : Siemens Atellica IM 1600

Department - Immunology

No. of test(s) performed - 5

Performed Test(s): **Vitamin B12 Cyanocobalamin, Vitamin D Total-25 Hydroxy, Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive)**

Description: The Siemens Healthineers system manufactured in Germany, combines AI-powered intelligent automation with advanced digitization to enhance performance and workflow efficiency. Utilizing acridinium ester-based chemiluminescence technology, it delivers highly sensitive and specific testing. With a throughput of up to 440 tests per hour and extensive automation, the system reduces manual handling, minimizes human error, and ensures reliable results for laboratories.

Frequency of Quality Control (QC) : **3 times a day** (Industry best and as per NABL guidelines)



Tosoh G11

Bio-Rad : Tosoh G11

Department - BIOCHEMISTRY HBA1C

No. of test(s) performed - 2

Performed Test(s): **HbA1c**

Description: The Tosoh Corporation fully automated HbA1c analyzer manufactured in Japan uses gold-standard cation exchange technology for direct, accurate, and precise measurement of HbA1c. It is NGSP certified, IFCC/DCCT traceable, and resistant to interferences from common haemoglobin variants.

Frequency of Quality Control (QC) : **Twice a day** (Industry best and as per NABL guidelines)



Horiba Yumi2500

HORIBA : Horiba Yumi2500

Department - Hematology

No. of test(s) performed - 26

Performed Test(s): **Complete Blood Count Advance**

Description: The fully automated high-end hematology analyzer manufactured in France, incorporates the patented Double Hydrodynamic Sequential System (DHSS) to ensure precise, linear cell flow for superior accuracy. It provides total WBC counts using impedance and flow cytometry, and supports detailed immature cell analysis—including immature granulocytes, monocytes, lymphocytes, and large immature cells—for effective sepsis screening and cancer pre-screening. The analyzer also features an optical platelet mode for enhanced clarity in cases of microcytosis, macro platelets, or RBC/PLT interferences, along with 360° homogeneous sample mixing to minimize manual intervention and preanalytical errors.

Frequency of Quality Control (QC) : **Twice a day** (Industry best and as per NABL guidelines)



Beckman AU700

Beckman Coulter : Beckman AU700

Department - Biochemistry

No. of test(s) performed - 1

Performed Test(s): **Blood Glucose Fasting**

Description: The Beckman Coulter state-of-the-art clinical chemistry analyzer manufactured in USA, delivers exceptional performance with a throughput of up to 800 tests per hour using spectrophotometry and 1,200 tests per hour with ISE, ensuring outstanding diagnostic accuracy and precision. Utilizing advanced spectrophotometric technology, it provides reliable and highly accurate results, supporting over 21 parameters that meet Six Sigma performance standards for superior laboratory efficiency and quality.

Frequency of Quality Control (QC) : **3 times a day** (Industry best and as per NABL guidelines)

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Suman Gupta

Booking ID : 16941687185 | Sample Collection Date : 17/Feb/2026

Suman Gupta,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

79

Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) - Ultrasensitive : 2.14 μ U/ml

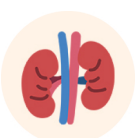
• Everything looks good



Cholesterol Total

139 mg/dl

• Everything looks good



Kidney Function

Serum Creatinine : 0.7 mg/dl

• Everything looks good



Vitamin D

64.91 ng/ml

• Everything looks good



HbA1c

5.3 %

• Everything looks good



Vitamin B12

337 pg/ml

• Everything looks good



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 24 U/L

• Everything looks good



Calcium Total

9 mg/dl

• Everything looks good



Iron studies

Serum Iron : 51 μ g/dl

• Concern



Complete Hemogram

Haemoglobin (HB) : 11.4 g/dL

• Concern



New Features

Report Summary

Suman Gupta

Booking ID 16941687185 | Sample Collection Date: 17/Feb/2026

Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

Here is a summary of your recent health test results and some suggestions:

Summary of Your Health Test Results

Hello Suman, thank you for sharing your recent health test results. It's completely understandable to feel concerned when some values are outside the typical range. Please know that many of these findings are common and often manageable with lifestyle adjustments and further medical guidance.

We've reviewed your results and noted a few areas that are a little different from the usual. These include some indicators related to your blood counts, liver function, and how your body is processing certain substances. It's important to remember that these are just numbers on a report, and they don't tell the whole story of your health. Many factors can influence these values, and we're here to help you understand what they mean in the context of your overall well-being.

Suggestions for Your Health

- 1. Focus on a Balanced Diet:** Incorporating a variety of nutrient-rich foods can be very beneficial. This includes plenty of fruits, vegetables, whole grains, and lean proteins. Reducing processed foods, excessive sugar, and unhealthy fats can also support your body's overall function.
- 2. Stay Hydrated:** Drinking an adequate amount of water throughout the day is crucial for many bodily processes, including liver function and blood circulation.
- 3. Gentle Physical Activity:** Regular, moderate exercise can positively impact blood counts and overall health. Aim for activities you enjoy, such as walking, swimming, or yoga, to help improve your well-being.
- 4. Stress Management:** Finding healthy ways to manage stress is important for your overall health. Techniques like deep breathing exercises, mindfulness, or engaging in hobbies can be very helpful.

HEALTH ANALYSIS

HISTORICAL CHARTS

Suman Gupta

Booking ID : 16941687185 | Sample Collection Date : 17/Feb/2026

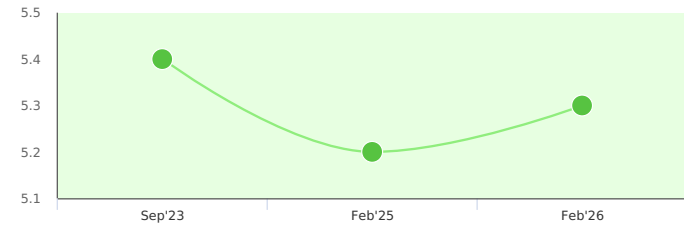
Glycated Hemoglobin (HbA1c)

Everything looks good

Your Latest result

5.3 %

17th Feb 2026



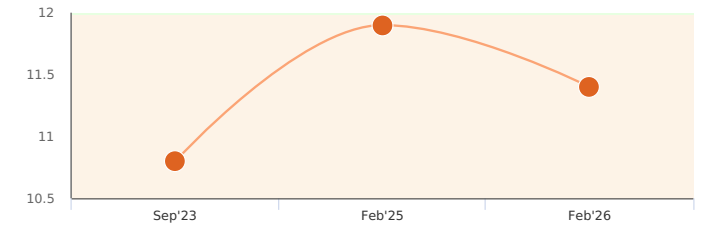
Hemoglobin Hb

Concern

Your Latest result

11.4 g/dL

17th Feb 2026



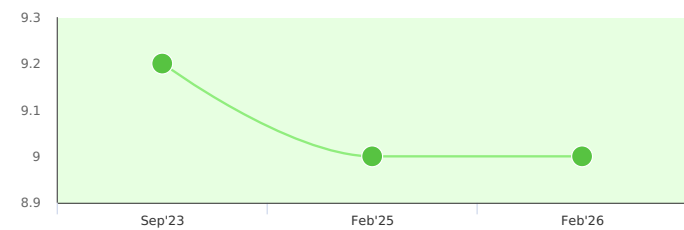
Calcium Total, Serum

Everything looks good

Your Latest result

9 mg/dL

17th Feb 2026



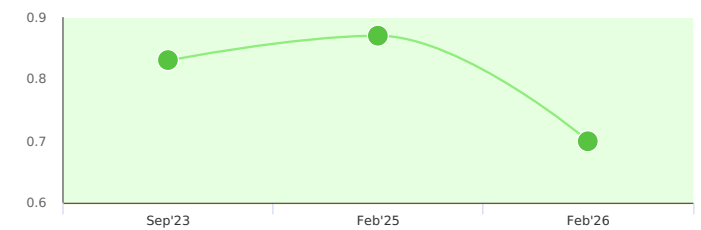
Creatinine, Serum

Everything looks good

Your Latest result

0.7 mg/dL

17th Feb 2026



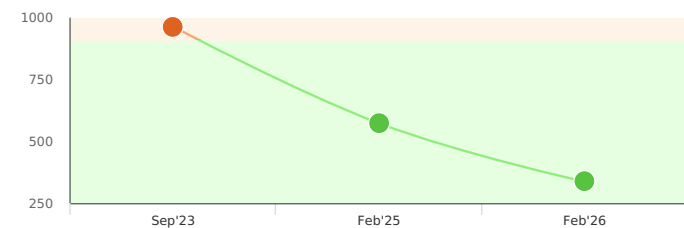
Vitamin B12 Cyanocobalamin

Everything looks good

Your Latest result

337 pg/ml

17th Feb 2026



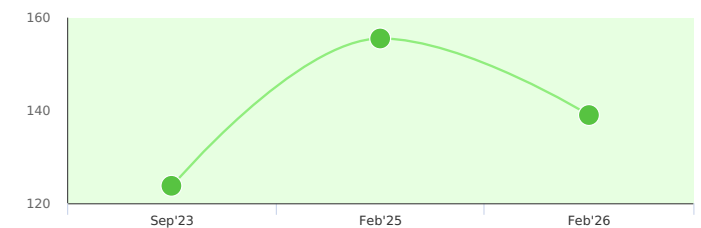
Cholesterol-Total, Serum

Everything looks good

Your Latest result

139 mg/dL

17th Feb 2026



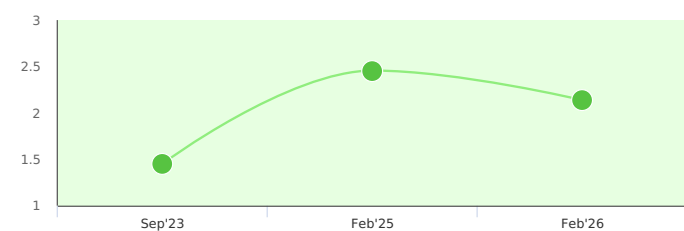
TSH Ultra - Sensitive

Everything looks good

Your Latest result

2.137 μ IU/ml

17th Feb 2026



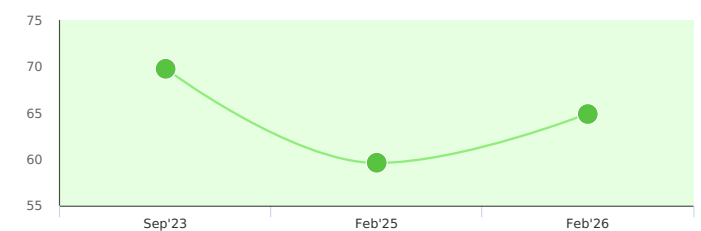
Vitamin D Total-25 Hydroxy

Everything looks good

Your Latest result

64.91 ng/ml

17th Feb 2026



HEALTH ANALYSIS

COMPARATIVE CHARTS

Suman Gupta

Booking ID : 16941687185 | Sample Collection Date : 17/Feb/2026

↑ Increase From Last Time ↓ Decrease From Last Time

Total Parameters :24
(As per latest result)

19 Everything Looks Good

1 Borderline

4 Concern

2 Years Ago

Test Name	Risk Area	Booking ID 8903691636 09 Sep 2023 06:00 AM	Latest Result 17 Feb 2026
Cholesterol-Total, Serum 0 -200 Normal Range	--	123.8 mg/dl	↑ 139 mg/dl Everything Looks Good
HDL Cholesterol Direct 40 -60 Normal Range	--	47.1 mg/dl	↑ 58.6 mg/dl Everything Looks Good
Triglycerides, Serum 0 -150 Normal Range	--	111.8 mg/dl	↓ 100 mg/dl Everything Looks Good
Alkaline Phosphatase, Serum 33 -98 Normal Range	--	85 u/L	↑ 92 u/L Everything Looks Good
GGTP (Gamma GT) 5 -38 Normal Range		37.5 u/L	↑ 39 u/L High (Concern)
Proteins, Serum 6.6 -8.3 Normal Range	--	6.9 gm/dl	↓ 6.82 gm/dl Everything Looks Good
SGOT/AST 3 -35 Normal Range		31 u/L	↑ 39 u/L High (Concern)
SGPT/ALT 3 -35 Normal Range	--	34 u/L	↓ 24 u/L Everything Looks Good
Hemoglobin Hb 12.0 -15.0 Normal Range		10.8 g/dl	↑ 11.4 g/dl Low (Concern)
Platelet Count Thrombocyte count 150 -410 Normal Range	--	166 10 ⁹ /μl	↓ 152 10 ⁹ /μl Everything Looks Good
T3, Total Tri Iodothyronine 0.60 -1.81 Normal Range	--	0.92 ng/ml	↓ 0.82 ng/ml Everything Looks Good
T4, Total Thyroxine 3.2 -12.6 Normal Range	--	8.7 ug/dl	↓ 5.8 ug/dl Everything Looks Good

HEALTH ANALYSIS

COMPARATIVE CHARTS

Suman Gupta

Booking ID : 16941687185 | Sample Collection Date : 17/Feb/2026

↑ Increase From Last Time ↓ Decrease From Last Time

Total Parameters :24
(As per latest result)

19 Everything Looks Good

1 Borderline

4 Concern

2 Years Ago

Test Name	Risk Area	Booking ID 8903691636 09 Sep 2023 06:00 AM	Latest Result 17 Feb 2026
TSH Ultra - Sensitive 0.55 - 4.78 Normal Range	--	1.46 μIU/ml	↑ 2.14 μIU/ml Everything Looks Good
Iron, Serum 60 - 180 Normal Range		-	51 ug/dl Low (Concern)
Calcium Total, Serum 8.8 - 10.6 Normal Range	--	9.2 mg/dl	↓ 9 mg/dl Everything Looks Good
Chlorides, Serum 101 - 109 Normal Range	--	106 mmol/L	↑ 107 mmol/L Everything Looks Good
Creatinine, Serum 0.6 - 1.3 Normal Range	--	0.83 mg/dl	↓ 0.7 mg/dl Everything Looks Good
Phosphorus-Inorganic, Serum 2.5 - 4.5 Normal Range	--	3.9 mg/dl	↓ 3.4 mg/dl Everything Looks Good
Sodium, Serum 136 - 146 Normal Range	--	142 mmol/L	142 mmol/L Everything Looks Good
Urea, Serum 17 - 43 Normal Range	--	22 mg/dl	↑ 33 mg/dl Everything Looks Good
Uric Acid, Serum 2.6 - 6 Normal Range	--	7.2 mg/dl	↓ 4.9 mg/dl Everything Looks Good
URINE KETONE Negative - Negative Normal Range	--	Negative	Negative
Glycated Hemoglobin (HbA1c) 4.2 - 5.7 Normal Range	--	5.4 %	↓ 5.3 % Everything Looks Good
Vitamin D Total-25 Hydroxy 30 - 100 Normal Range	--	69.76 ng/ml	↓ 64.91 ng/ml Everything Looks Good

Patient Name	: Suman Gupta	Barcode	: E4887357	
Age/Gender	: 61Y 0M 0D /Female	Sample Collected On	: 17/Feb/2026 06:56AM	
Order Id	: 16941687185	Sample Received On	: 17/Feb/2026 02:27PM	
Referred By	: Self	Report Generated On	: 17/Feb/2026 04:12PM	
Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY HBA1C

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycosylated Hemoglobin

Hba1c (Glycosylated Hemoglobin)	5.30	%	4.2 - 5.7
Method: HPLC			
Average Estimated Glucose - plasma	105.41	mg/dl	
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7

5.7 – 6.4

>= 6.5

Age > 19 Years

Goals of Therapy: < 7.0

Actions Suggested: >8.0

Age < 19 Years

Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
- HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
- Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapsone, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
- HbA1c may be increased in patients with polycythemia or post-splenectomy.
- Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test.
- Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
- HbA1c target in pregnancy is to attain level <6 % .
- HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
- Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023
- The presence of some hemoglobinopathies/hemoglobin variants may be missed by some platforms used for measuring HbA1c values. In these cases, the HbA1c result may vary or give deviant results, depending on the platform used. Alternative measures of glycemic control like fructosamine or glycated albumin may be considered in such cases.
- In presence of hemoglobinopathy/hemoglobin variants in homozygous/compound heterozygous conditions alternative forms of testing such as fructosamine or glycated albumin may be considered.



Dr. Priyanka Gothwal
MBBS, MD Pathology
Consultant Pathologist
HN-28938, Healthians Labs



SIN No:E4887357

Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582)

Patient Name	: Suman Gupta	Barcode	: E4887357	
Age/Gender	: 61Y 0M 0D /Female	Sample Collected On	: 17/Feb/2026 06:56AM	
Order Id	: 16941687185	Sample Received On	: 17/Feb/2026 02:28PM	
Referred By	: Self	Report Generated On	: 17/Feb/2026 08:23PM	
Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Fasting Blood Sugar

Glucose, Fasting	83.2	mg/dl	70 - 100
Method: Hexokinase			
Machine: BECKMAN COULTER DXC 700 AU			

American Diabetes Association Reference Range :

Normal	: < 100 mg/dl
Impaired fasting glucose(Prediabetes)	: 100 - 125 mg/dl
Diabetes	: >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include:

- Diabetes mellitus ,Hemochromatosis ,Cushing syndrome ,Acromegaly and gigantism.
- Increased circulating epinephrine such as in pheochromocytoma and adrenalin injections
- Acute pancreatitis
- Chronic pancreatitis

Conditions that cause low blood glucose level include :

- Pancreatic disorders : Islet cell tumor , pancreatitis
- Hepatic disease (diffuse severe disease)
- Endocrine disorders : hypopituitarism, Addison's disease ,hypothyroidism
- Alcoholism
- Malnutrition



Dr. Preeti
MBBS, MD Biochemistry
Consultant Biochemist
DMC - 97534, Healthians Labs



SIN No:E4887357



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Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Rheumatoid Factor (RA) - Quantitative - Serum

RHEUMATOID FACTOR	<10	IU/mL	<14
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Method: Latex Particle Immunoturbidimetric

Machine: BECKMAN COULTER AU 5800

Rheumatoid factor is an immunoglobulin present in serum of 50 -95% of adults with Rheumatoid arthritis(RA).It appears in serum and synovial fluid several months after onset of RA and is present upto years after therapy

Use

- Assisting in the diagnosis of RA ,especially when clinical diagnosis is difficult

Increased in : RA ,Chronic hepatitis , chronic viral infection ,Cirrhosis, SLE, Scleroderma ,syphilis ,Infectious mononucleosis, TB ,Sjogren syndrome , Sarcoidosis , malaria, Leishmaniasis



Dr. Preeti
MBBS, MD Biochemistry
Consultant Biochemist
DMC - 97534, Healthians Labs



SIN No:E4887357



Patient Name : Suman Gupta
Age/Gender : 61Y 0M 0D /Female
Order Id : 16941687185
Referred By : Self
Customer Since : 09/Sep/2023
Sample Type : Serum

Barcode : E4887357
Sample Collected On : 17/Feb/2026 06:56AM
Sample Received On : 17/Feb/2026 02:28PM
Report Generated On : 17/Feb/2026 07:42PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Liver Function Test (LFT)

Serum Bilirubin, (Total) Method: DPD Machine: BECKMAN COULTER AU 5800	1.62	mg/dl	0.3 - 1.2
Serum Bilirubin, (Direct) Method: DPD Machine: BECKMAN COULTER AU 5800	0.34	mg/dl	0 - 0.2
Serum Bilirubin, (Indirect) Method: Calculated	1.28	mg/dl	0.0 - 0.8
Aspartate Aminotransferase (AST/SGOT) Method: UV without P5P Machine: BECKMAN COULTER AU 5800	39.00	U/L	3 - 35
Alanine Aminotransferase (ALT/SGPT) Method: UV without P5P Machine: BECKMAN COULTER AU 5800	24	U/L	3 - 35
Alkaline Phosphatase (ALP) Method: AMP Buffer Machine: BECKMAN COULTER AU 5800	92.00	U/L	33-98
Gamma Glutamyl Transferase (GGT) Method: IFCC Machine: BECKMAN COULTER AU 5800	39.0	U/L	5- 38
Serum Total Protein Method: BIURET Machine: BECKMAN COULTER AU 5800	6.82	gm/dl	6.6 - 8.3
Serum Albumin Method: BROMOCRESOL GREEN Machine: BECKMAN COULTER AU 5800	3.91	g/dl	3.5 - 5.2
Serum Globulin Method: Calculated	2.91	gm/dl	3.0 - 4.2
Albumin/Globulin Ratio Method: Calculated	1.34	Ratio	1.2 - 2.5
SGOT/SGPT Ratio Method: Calculated	1.62	Ratio	0.7 - 1.4

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels are a result of increased bilirubin production (e.g hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (e.g.; obstruction and hepatitis) and abnormal

Preeti

Dr. Preeti
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DEPARTMENT OF BIOCHEMISTRY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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bilirubin metabolism (e.g; hereditary and neonatal jaundice) .

Conjugated (direct) bilirubin is elevated in conditions like- Hereditary disorders(Dubin Johnson syndrome, Rotor syndrome),Hepatocellular damage(e.g –viral ,toxic ,alcohol ,drugs) ,biliary duct obstruction (extrahepatic or intrahepatic), Infiltrations ,space occupying lesions(e.g metastasis, abscess , granuloma , amyloidosis. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.

ALT is a liver specific enzyme commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.

Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc.

Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Obstructive liver disease, high alcohol consumption and use of enzyme-inducing drugs lead to raised GGT levels .

Serum total protein measures the total amount of protein in serum. It is largely comprised of albumin and globulins. Increased levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, multiple myeloma, Waldenstrom's disease. Decreased levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic Syndrome.

Albumin is the most abundant protein in the serum and is produced in the liver. Low serum albumin levels (hypoalbuminemia) can be caused by: Liver diseases like liver cirrhosis, nephrotic syndrome, protein-losing enteropathy, burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting .

Globulins are increased in most liver diseases , in chronic inflammatory diseases and neoplastic diseases.

Please note that there is a change in the Biological Reference Interval for Alkaline Phosphatase (ALP) parameter. (Dated: 11.02.2026).



Dr. Preeti
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DMC - 97534, Healthians Labs



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Referred By	: Self	Report Generated On	: 17/Feb/2026 07:42PM
Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓
Sample Type	: SERUM	Report Status	: Final Report

DEPARTMENT OF BIOCHEMISTRY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
Iron study			
Serum Iron	51.0	ug/dl	60 - 180
Method: TPTZ			
Machine: BECKMAN COULTER AU 5800			
UIBC	343.00	ug/dl	155 - 355
Method: Nitroso-PSAP			
Machine: BECKMAN COULTER AU 5800			
Serum Total Iron Binding Capacity (TIBC)	394	µg/dl	250 - 400
Method: FE+UIBC (saturation with iron)			
Transferrin Saturation %	12.94	%	15 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin, estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein-calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

Preeti

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SIN No: E4887357



Patient Name	: Suman Gupta	Barcode	: E4887357	
Age/Gender	: 61Y 0M 0D /Female	Sample Collected On	: 17/Feb/2026 06:56AM	
Order Id	: 16941687185	Sample Received On	: 17/Feb/2026 02:28PM	
Referred By	: Self	Report Generated On	: 17/Feb/2026 08:23PM	
Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓	
Sample Type	: SERUM	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Kidney Function Test Advance

Serum Creatinine Method: Modified Jaffe, Kinetic Machine: BECKMAN COULTER AU 5800	0.70	mg/dl	0.6 - 1.3
GFR, ESTIMATED Method: Calculated (CKD-EPI)	98.33	mL/min/1.73m2	
Serum Uric Acid Method: Uricase PAP Machine: BECKMAN COULTER AU 5800	4.9	mg/dl	2.6-6.0
Serum Calcium Method: ARSENAZO III Machine: BECKMAN COULTER AU 5800	9.0	mg/dl	8.8 - 10.6
Serum Phosphorus Method: PHOSPHOMOLYBDATE COMPLEX Machine: BECKMAN COULTER AU 5800	3.4	mg/dl	2.5 - 4.5
Serum Sodium Method: ISE (Indirect) Machine: BECKMAN COULTER AU 5800	142	mmol/L	136 - 146
Serum Chloride Method: ISE (Indirect) Machine: BECKMAN COULTER AU 5800	107	mmol/L	101 - 109
Blood Urea Method: GLDH,Kinetic assay Machine: BECKMAN COULTER AU 5800	33.0	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	15.4	mg/dl	8 - 20
Bun/Creatinine Ratio Method: Calculated	22.03	Ratio	
Urea/Creatinine Ratio	47.14		

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.

Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.



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Test Name	Value	Unit	Bio. Ref Interval
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Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout. Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR . Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases. Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (>=20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factor like race muscle mass, diet and certain drugs in such case EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85


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DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol Method: CHO-POD (Trac. Abel-Kendall) Machine: BECKMAN COULTER AU 5800	139.0	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: GPO-POD Machine: BECKMAN COULTER AU 5800	100.0	mg/dL	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : > 500
Serum HDL Cholesterol Method: ENZYMATIC IMMUNOINHIBITION Machine: BECKMAN COULTER AU 5800	58.6	mg/dL	40 - 60
LDL Cholesterol Calculated Method: Calculated	60.40	mg/dL	Optimal : <100 near /above Optimal:100 - 129 Borderline High: 130- 159 High : 160 - 189 Very High :>=190
VLDL Cholesterol Calculated Method: Calculated	20	mg/dl	<30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	2.37	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	1.03	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio Method: Calculated	0.97	Ratio	Optimal->0.4 Moderate-0.4 to 0.3 High-<0.3
Non-HDL Cholesterol Method: Calculated	80.4	mg/dL	0.0 - 160.0

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations. Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone, vitamin D, estrogens, glucocorticoids and mineralocorticoids.

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DEPARTMENT OF BIOCHEMISTRY

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HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease.

LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary heart disease.

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. Healthians labs report biological reference intervals (normal ranges) in accordance with the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.

Updated risk stratification approach recommended by the Lipid Association of India (2023)			
Risk Category			
Extreme Risk	Category A: ASCVD with ≥ 1 feature of high-risk group ●CACS ≥300 ●Homozygous FH		
	Category B: ASCVD with ≥ 1 feature of very high-risk group or ● Recurrent ACS or ● Polyvascular disease or ●Homozygous FH		
	Category C: Recurrent ASCVD event despite LDL-C around 30 mg/dL. These patients require special consideration.		
Very High Risk (VHR)	●Diabetes with target organ damage ● Diabetes with ≥2 major ASCVD risk factors ●CACS 100–299 or >75 percentile if CACS 1–99 ●≥2 high risk features ●Established ASCVD (obstructive or non-obstructive) ●Heterozygous FH or LDL-C ≥190 mg/dL		
High Risk (HR)	●>3 major ASCVD risk factors, or ●LDL-C 160–189 mg/dL or ●Non-HDL-C 190–219 mg/dL or ●Diabetes with 0–1 major ASCVD risk factor ●2 major ASCVD factors + ≥1 risk modifier or ●Any 1 high-risk feature.		
Moderate Risk (MR)	●2 major ASCVD risk factors, or ●LDL-C 130–159 mg/dL or ●Non-HDL-C 160–189 mg/dL or ●Low-risk group + ≥1 risk modifier or lifetime ASCVD risk >30%		
Low Risk (LR)	●0–1 major ASCVD risk factor and ●LDL-C 100–129 mg/dL and ●Non-HDL-C 130–159 mg/dL, and ●Lifetime CVD risk <30%		
Major Risk ASCVD (Atherosclerotic Cardiovascular Disease) Risk Factors			
●Age ≥45 years in males and ≥55 years in females ●Current cigarette smoking or tobacco use ●High blood pressure ●Low HDL-C			
Treatment targets for lipid lowering therapy for various atherosclerotic cardiovascular disease risk groups			
Risk Groups	Treatment Targets		
	LDL-C, mg/dl (Primary Target)	Non-HDL-C,mg/dl(Coprimary Target)	Apo-B, mg/dl (Secondary Target)
Low risk group	<100	<130	<90
Moderate risk group	<100 (optional <70)	<130 (optional <100)	<90
High risk group	<70	<100	<80
Very high risk	<50	<80	<65
Extreme -risk-group- Category (A)	<50 (optional ≤30)	<80 (optional ≤60)	<65
Extreme -risk-group- Category (B)	≤30	≤60	<50
Extreme -risk-group- Category (C)	10-15	40-45	-----

Reference: Journal of Clinical Lipidology, Vol-18 No-3. May/June 2024


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Sample Type	: URINE	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Pale Yellow		Pale Yellow
Method: Visual			
Volume	15.00	mL	
Method: Visual			
Appearance	Slightly Hazy		Clear
Method: Visual			

CHEMICAL EXAMINATION

Specific Gravity	1.010		1.001 - 1.035
Method: Dipstick-Ion exchange			
Machine: URIPLUS_600			
pH	5.5		4.5 - 7.5
Method: Dipstick-Double indicator			
Machine: URIPLUS_600			
Glucose	Negative		Negative
Method: Dipstick-oxidase peroxidase			
Machine: URIPLUS_600			
Urine Protein	Negative		Negative
Method: Dipstick-Bromophenol blue			
Machine: URIPLUS_600			
Ketones	Negative		Negative
Method: Sodium nitroprusside			
Machine: URIPLUS_600			
Urobilinogen	Normal		Normal
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS_600			
Bilirubin	Negative		Negative
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS_600			
Nitrite	Negative		Negative
Method: Dipstick-Griess test			
Machine: URIPLUS_600			
Blood	Negative		Nil



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DEPARTMENT OF CLINICAL PATHOLOGY

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Test Name	Value	Unit	Bio. Ref Interval
Method: Dipstick-Peroxidase Machine: URIPLUS_600			
Leucocyte Esterase	2+		Nil
Method: Dipstick- Esterase Machine: URIPLUS_600			
MICROSCOPIC EXAMINATION			
Pus Cells	15-20	/HPF	0 - 5
Method: Microscopic Examination			
Epithelial cells	2-3	/HPF	0 - 5
Method: Microscopic Examination			
RBCs	Nil	/HPF	Nil
Method: Microscopic Examination			
Casts	Nil		Nil
Method: Microscopic Examination			
Crystals	Nil		Nil
Method: Microscopic Examination			
Bacteria	Absent		Absent
Method: Microscopic Examination			
Yeast Cell	Absent		Absent
Method: Microscopic Examination			
Others (Non Specific)	Nil		NIL
Method: Microscopic Examination			

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra.

Ketonuria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract



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Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582)

Patient Name	: Suman Gupta	Barcode	: E4887357	
Age/Gender	: 61Y 0M 0D /Female	Sample Collected On	: 17/Feb/2026 06:56AM	
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DEPARTMENT OF CLINICAL PATHOLOGY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.



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Referred By	: Self	Report Generated On	: 17/Feb/2026 02:53PM	
Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓	
Sample Type	: SODIUM CITRATE	Report Status	: Final Report	

DEPARTMENT OF COAGULATION

Test Name	Value	Unit	Bio. Ref Interval
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Prothrombin Time (PT) INR

Prothrombin Time **25.00** Sec. 8.9 - 13.3

Method: Optical Method (Light Scattering)

Machine: WERFEN ACL TOP 350

Control (MNPT) **11.10** Sec

Method: Calculated

Ratio **2.25**

Method: Calculated

Machine: WERFEN ACL TOP 350

International Normalized Ratio (INR) **2.20** 0.80 - 1.16

Method: Calculated

Machine: WERFEN ACL TOP 350

Prothrombin time (PT) is used to monitor patients on anti clotting drugs as well as to diagnose clotting disorders. A PT test also called as an INR (International Normalized Ratio) is a way of standardising the results of PT test, no matter the testing method.

INR is used to monitor patients on anticoagulation therapy.

INR of 2.0 – 3.0 is usually targeted for patients on warfarin therapy. A high INR indicates risk of bleeding, while a low INR suggests a risk of developing clot.

Causes of prolonged PT:

- Factor II deficiency.
- Liver diseases.
- Vitamin K deficiency.
- Warfarin.

Causes of prolonged PT & APTT:

- Vitamin K deficiency.
- Liver diseases.
- Factor II, VIII, II and fibrinogen deficiency.
- DIC.
- Massive blood transfusion.

Please note targeted INR may vary with respect to the clinical condition for which the anticoagulant therapy is used. Patient should contact their clinician for further advise.



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Patient Name : Suman Gupta
Age/Gender : 61Y 0M 0D /Female
Order Id : 16941687185
Referred By : Self
Customer Since : 09/Sep/2023
Sample Type : WHOLE BLOOD EDTA

Barcode : E4887357
Sample Collected On : 17/Feb/2026 06:56AM
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DEPARTMENT OF HAEMATOLOGY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count Advanced

Haemoglobin (HB) Method: Spectrophotometry Machine: Yumizen H2500	11.4	g/dL	12.0-15.0
Total Leucocyte Count (TLC) Method: Impedance Machine: Yumizen H2500	4.2	10 ³ /uL	4.0-10.0
Hematocrit (PCV) Method: Calculated Machine: Yumizen H2500	34.7	%	36.0-46.0
Red Blood Cell Count (RBC) Method: Impedance Machine: Yumizen H2500	3.87	10 ⁶ /μl	3.80-4.80
Mean Corp Volume (MCV) Method: Derived from RBC Histogram Machine: Yumizen H2500	89.7	fL	83.0-101.0
Mean Corp Hb (MCH) Method: Calculated Machine: Yumizen H2500	29.4	pg	27.0-32.0
Mean Corp Hb Conc (MCHC) Method: Calculated Machine: Yumizen H2500	32.8	g/dL	31.5-34.5
RDW - CV Method: Derived from RBC Histogram Machine: Yumizen H2500	14.9	%	11.6-14.0
RDW - SD Method: Derived from RBC Histogram Machine: Yumizen H2500	37.20	fL	39.0-46.0
Mentzer Index Method: Calculated	23.18	Ratio	
RDWI Method: Calculated	345.36	Ratio	
Green and king index Method: Calculated	105	Ratio	

Differential Leucocyte Count

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DEPARTMENT OF HAEMATOLOGY

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Test Name	Value	Unit	Bio. Ref Interval
Neutrophils Method: Flow-Cytometry DHSS Machine: Yumizen H2500	56.6	%	40 - 80
Lymphocytes Method: Flow-Cytometry DHSS Machine: Yumizen H2500	34.7	%	20-40
Monocytes Method: Flow-Cytometry DHSS Machine: Yumizen H2500	7.0	%	02 - 10
Eosinophils Method: Flow-Cytometry DHSS Machine: Yumizen H2500	1.6	%	01 - 06
Basophils Method: Impedance Machine: Yumizen H2500	0.1	%	00 - 02
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC) Method: Calculated	2.39	10 ³ /uL	2.0-7.0
Absolute Lymphocyte Count (ALC) Method: Calculated	1.46	10 ³ /uL	1.0-3.0
Absolute Monocyte Count Method: Calculated	0.30	10 ³ /uL	0.2-1.0
Absolute Eosinophil Count (AEC) Method: Calculated	0.07	10 ³ /uL	0.02-0.5
Absolute Basophil Count Method: Calculated	0	10 ³ /uL	0.02 - 0.10
Platelet Count(PLT) Method: Impedance Machine: Yumizen H2500	152	10 ³ /μl	150-410
MPV Method: Derived from PLT Histogram Machine: Yumizen H2500	12.2	fL	7 - 9
NLR (Neutrophil Lymphocyte Ratio) Method: Calculated	1.64	Ratio	
PLR (Platelet Lymphocyte Ratio)	104	Ratio	

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DEPARTMENT OF HAEMATOLOGY

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Method: Calculated

RESULT ARE VERIFIED ON SMEAR EXAMINATION.

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron deficiency anaemia is more likely.

Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <220 more likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

Calculated parameters are either derived from Impedance measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

The neutrophil to lymphocyte ratio (NLR) is considered to be a sensitive indicator of infection, inflammation and sepsis, and a robust prognostic marker of disease severity.

The platelet-to-lymphocyte ratio (PLR) is a marker revealing shifts in platelet & lymphocyte cell counts due to acute inflammatory and pro-thrombotic states.



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SIN No:E4887357

Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582)

Patient Name	: Suman Gupta	Barcode	: E4887357	
Age/Gender	: 61Y 0M 0D /Female	Sample Collected On	: 17/Feb/2026 06:56AM	
Order Id	: 16941687185	Sample Received On	: 17/Feb/2026 02:28PM	
Referred By	: Self	Report Generated On	: 17/Feb/2026 04:24PM	
Customer Since	: 09/Sep/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthy India 2026 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin B12

VITAMIN B12	337	pg/ml	211 - 912
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Method: CLIA

Machine: Siemens Atellica IM1300

Interpretation

Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine from homocysteine and 2) the conversion of methyl malonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. It can also lead to macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC mean corpuscular volume (MCV) may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of Vitamin B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for Vitamin B12 deficiency at the cellular level is the assay for methyl malonic acid (MMA). If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

The Bio Ref Interval for Vitamin B12 has been changed on Beckman Coulter UniCel Dx1800 with effect from 18.2.25.

Preeti

Dr. Preeti
MBBS, MD Biochemistry
Consultant Biochemist
DMC - 97534, Healthians Labs



SIN No: E4887357



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Test Name	Value	Unit	Bio. Ref Interval
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Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	64.91	ng/ml	30 - 100
Method: CLIA			
Machine: Siemens Atellica IM1300			

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL), Adult	VITAMIN D 25 HYDROXY (ng/mL), Pediatric
DEFICIENCY	<20	<15
INSUFFICIENCY	20 - 30	15 - 20
SUFFICIENCY	30 - 100	20 - 100

Vitamin D is a steroid hormone known for its important role in regulating body levels of calcium and phosphorus and in the mineralization of bone

Uses:

- Diagnosis of Vitamin D deficiency
- Differential diagnosis of causes of rickets and Osteomalacia
- Monitoring Vitamin D replacement therapy
- Diagnosis of hypervitaminosis D

Increased in:

- Vitamin D intoxication
- Excessive exposure to sunlight

Decreased in:

- Malabsorption
- Steatorrhoea
- Dietary osteomalacia
- Thyrotoxicosis
- Coeliac disease
- Inflammatory bowel disease
- Rickets
- Pancreatic insufficiency

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Thyroid Profile (Total T3,T4, TSH)

Tri-Iodothyronine (T3, Total)	0.82	ng/ml	0.60-1.81
Method: CLIA			
Machine: Siemens Atellica IM1300			
Thyroxine (T4, Total)	5.80	ug/dl	3.2-12.6
Method: CLIA			
Machine: Siemens Atellica IM1300			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	2.137	μIU/ml	0.55-4.78
Method: CLIA			
Machine: Siemens Atellica IM1300			

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians

recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

*** End Of Report ***

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Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582 | NABL Accreditation Certificate Number MC-4245)

Terms & Conditions:

- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of $\pm 50\%$, hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.
- 13) This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- 14) All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- 15) This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- 16) It is strongly recommended to take required precautions for allergic reactions or sensitivities.
- 17) Authorised partner labs as mentioned for certain tests are as below:
 - HL/PL/001- Metropolis Healthcare Ltd
 - HL/PL/002- Thyrocare technologies Limited
 - HL/PL/003- Lifecell International Pvt. Ltd. - Laboratory Services
 - HL/PL/004- Modern Diagnostic & Research Centre

ADVISORY

Health Advisory

Suman Gupta

Booking ID : 16941687185 | Sample Collection Date : 17/Feb/2026

20.81 Body Mass Index

5'1" Height (ft/in)

50 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

0cm



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED NUTRITION

SUGGESTED NUTRITION

Do's

- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Include fruits like apples, berries and melons in your diet
- Have dates and figs
- Take vitamin C rich foods like citrus fruits, strawberries and green, leafy vegetables

Dont's

- Avoid flavoured and seasoned foods
- Decrease intake of colas and sugary drinks
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Limit intake of salt
- Avoid refined carbs, processed foods
- Avoid the use of oil and avoid sauces and dressings

SUGGESTED LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Have breakfast early in morning and a light high fiber snack for dinner
- Maintain ideal weight
- Plan a healthy routine and have food at the same time everyday

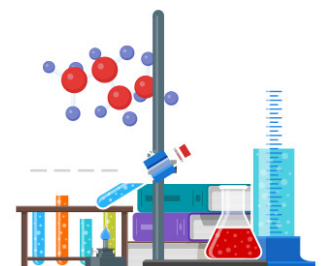
Dont's

- Avoid overexertion without having food or drink
- Avoid strenuous exercises
- Avoid smoking and alcohol

SUGGESTED FUTURE TESTS

SUGGESTED FUTURE TESTS

- Complete Hemogram - **Every 2 Month**
- Peripheral Smear Examination By Pathologist - **Every 2 Month**
- Iron Studies - **Every 1 Month**



HEALTH ADVISORY

Suggestions for Health & Well-being

Suman Gupta

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PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is **20.81**, which is on a normal side.

Please follow recommended diet and maintain a healthy lifestyle and try to keep your BMI within the desired range. Keeping the right BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART



SUGGESTED BMI

Supplement Suggestions

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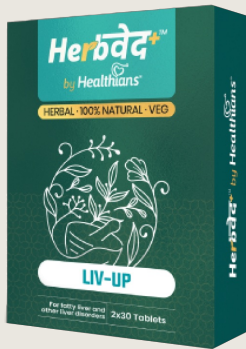
Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Bilirubin Total, Serum	LIV-UP
Iron, Serum	IRON POWER

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



LIV-UP

De-toxify your body with a healthier liver.

LIV-UP is a scientifically formulated and clinically proven all-natural supplement that takes care of your liver and its functions. This ayurvedic supplement keeps your liver cool, and optimally functioning, thus promoting healthy digestion. Take the all-natural road to robust liver health with LIV-UP.

Untreated or unmanaged liver issues can cause grave and even lethal complications, which include:

- Liver Infections
- Liver Cancer
- Liver Failure
- Elevated Blood Toxin Levels
- Liver Cirrhosis

Infused with the ages-proven goodness of all-natural ingredients, LIV-UP is the perfect supplement to promote and maintain good liver health, without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in LIV-UP present the following benefits:

Methi Dana

Reducing the risk of developing fatty liver disease.

Triphala

Promotes liver function & boosts immunity

Ginger

Supports liver health & prevents liver inflammation

Yellow Myrobalan

Keeps the liver cool & promotes optimal functioning

Kulki

Reduces inflammation & protects the liver from injury



IRON POWER

Your all natural weapon against Anaemia

IRON-POWER is a scientifically formulated and clinically proven all-natural supplement that helps you replenish iron in your system and significantly reduce the risk of anemia. This ayurvedic supplement increases blood production and iron levels to keep your body functioning optimally. Keep overall health robust and treat iron deficiency with IRON-POWER

It's widely known that iron is crucial for your health. A lack of iron intake or iron deficiency can cause serious health issues, such as:

- Heart problems
- Growth issues in children
- Pregnancy complications
- Depression
- Increased infection risk

Infused with the ages-proven goodness of all-natural ingredients, IRON-POWER is the perfect supplement to enhance and maintain your iron levels without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in IRON-POWER present the following benefits:

Orange Peel Extract

Rich in vitamin C. Helps maintain iron levels

WHEN GOOD FOOD MAKES YOU FEEL BAD,

it's time to find out why.

**FOOD
INTOLERANCE TEST**

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ONLY**

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About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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